CASE PRESENTATION

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3RD YEAR

BGS GIMS

CASE PRESENTATION :

I present to you a case of a 6year old boy named ABC , currently in his 2nd standard , residing in Vijaynagar belonging to a lower middle class family , had come into the opd on the 4th of august at 12pm , accompanied by his mother.

CHIEF COMPLAINTS:

- Cough – 2 weeks
- Decreased hearing in both ears - 3 days
- Ear pain – 1day



HOPI:

* The child was apparently normal 2 weeks back when he developed cough, that was insidious in onset and gradually progressive, it is a brassy type of cough , a productive cough(sputum: mucopurulent, yellow , not blood tinged) , no diurnal variation , increases to some extent while talking and decreases on drinking hot water and taking a cough syrup, no postural variation.

It is associated with sore throat and fever (insidious , moderate ,intermittent , relieved on medication, no aggrevating factors, no rashes)

Not associated with weight loss.

There was decreased oral intake of solid food due to soreness in the throat and pain during swallowing.

. The child also complains of decreased hearing since 3 days that was sudden in onset and fast in progression , it initially started in right ear and has involved the left ear also. The child could initially hear the conversation voice in quiet place ,but now needs shouting(moderate). This was associated with a mild ear ache

The child now complains of ear pain since 1 day , which is sudden in onset and rapidly progressive, non radiating, throbbing in nature , no aggrevating and relieving factors , it is a continous pain. This pain is associated with fever (which according to the mother has increased from the previous existing fever). It is not associated with any discharge .The child has disturbed sleep due to the pain. The mother also complains of the child becoming restless. It is due to this pain that the mother has got the child to the hospital .

The mother gives a history of a recent travel to their village 3 weeks back , where they shared their room with many family members.

No h/o trauma & foreign body insertion into the ears

No h/o ringing sensation in ear, giddiness, headache, vomiting ,pain behind the ear , facial pain

No h/o breathlessness & wheezing

No h/o missing any vaccines

No h/o significant weight loss

No h/o decreased appetite

No h/o nasal obstruction , nasal discharge , facial pain, nasal allergy symptoms

PAST HISTORY :

- No similar history in the past
- No h/o direct trauma to the ears
- No h/o Asthma , epilepsy

PEDIATRIC HISTORY:

Term baby , normal delivery , birth wt- 3kg , cried immediately after birth ,the antenatal and postnatal periods were normal , the milestones are reached in accordance with age , the child has been immunized appropriately .

FAMILY HISTORY :

Nothing significant

PERSONAL HISTORY:

- Diet : Vegetarian
- Appetite: Decreased solid food intake
- Sleep :Disturbed due to pain
- Bowel and bladder movements: Normal

Provisional Diagnosis :

On the basis of history the probable differential diagnosis could be :

- Acute suppurative otitis media
- Acute otitis externa
- Otitis media with effusion

Physical Examination

GENERAL PHYSICAL EXAMINATION:

The patient is conscious, co-operative ,and is well oriented to time , place and person. The patient is moderately built and is well nourished. The patient appears tired, irritable and keeps tugging his ears.

- Pallor, icterus , cyanosis , clubbing , edema are absent. Lymphadenopathy is seen in LEVEL 1,2,5 and 6.

VITAL SIGNS:

- **BP**: 86/60 mm hg
- **Respiratory rate:30** breaths /min
- **Temperature**: 39C/102.2F
- **Pulse** : Rate:68bpm

GENERAL SYSYTEMIC EXAMINATION:

CVS: S1 S2 heard , no murmurs found

CNS: No focal deficit detected

RS :Normal vesicular breath sounds heard

GIT: Normal

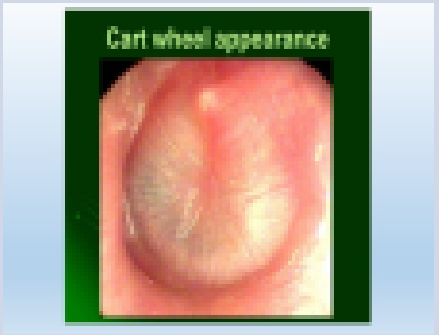
GUT: Normal

LOCAL ENT EXAMINATION:

The local examination was done with the consent of the child's mother ,with the patient seated on a chair opposite the examiner. The examination is done using a Bull's eye lamp and a head mirror.

EXA

EXAMINATION	RIGHT EAR	LEFT EAR
PINNA	NORMAL	NORMAL
PREAURICULAR AREA	NORMAL	NORMAL
POSTAURICULAR AREA	NORMAL	NORMAL
MASTOID TENDERNESS	ABSENT	ABSENT
EAC	NORMAL	NORMAL

EXAMINATION	RIGHT EAR	LEFT EAR
TYMPANIC MEMBRANE 	Appears red and slightly bulging . Leash of blood vessels along handle of malleus and at periphery seen: Cart wheel appearance .No perforation	Appears red and slightly bulging . Leash of blood vessels along handle of malleus and at periphery seen: Cart wheel appearance. No perforation
MIDDLE EAR	----------	----------
FACIAL NERVE	Normal	Normal
FISTULA TEST	NEGATIVE	NEGATIVE
RINNE TEST	NEGATIVE	NEGATIVE
WEBER TEST	CENTRALIZED	CENTRALIZED

EXAMINATION OF NOSE AND SINUSES:

- **External appearance**: Normal
- **Vestibule**: Normal
- **Anterior rhinoscopy**: The nasal passage , septum , floor of nose , lateral wall appear to be normal.
- **Posterior rhinoscopy** : NOT DONE
- **Functional examination of nose** : Normal
- **Paranasal sinuses tenderness**: Absent

Examination of Nasopharynx :

Posterior Rhinoscopy : NOT DONE

Cranial nerve examination: Normal (9,10,11)

EXAMINATION OF ORAL CAVITY:

- **Lips** : Normal
- **Buccal mucosa** : Normal
- **Gums and teeth**: Normal
- **Hard palate**: Normal
- **Tongue**: Normal
- **Floor of mouth**: Normal
- **Retromolar trigone** : Normal

EXAMINATION OF OROPHARYNX:

TONSILS:

Grade 1 tonsils , Congested

Ulcers , membrane, mass and bulge are not seen

PILLARS: * Anterior: Congested

* Posterior : Congested

SOFT PALATE: Appears congested

Deviation is not seen

POSTERIOR PHARYNGEAL WALL: congested

BASE OF TONGUE AND VALLECULA : On indirect laryngoscopy the lingual tonsils appear to be enlarged , vallecula – Normal

EXAMINATION OF LARYNX AND LARYNGOPHARYNX:

Larynx:

Epiglottis, aryepiglottic folds arytenoids, ventricular bands , ant commisure , post. Commisure , subglottis and rings of trachea appear NORMAL

Laryngopharynx:

Pyriform fossa , post cricoid region, posterior wall of laryngopharynx appear to be hyperaemic and congested.

EXAMINATION OF HEAD AND NECK LYMPH NODES:

Level 1 , 2 , 5 and 6 are enlarged upto 1cm in size , they are tender , around 4 to 5 nodes are enlarged in each of these levels.

Level 3,4 and 7 see no lymphadenopathy.

DIAGNOSIS:



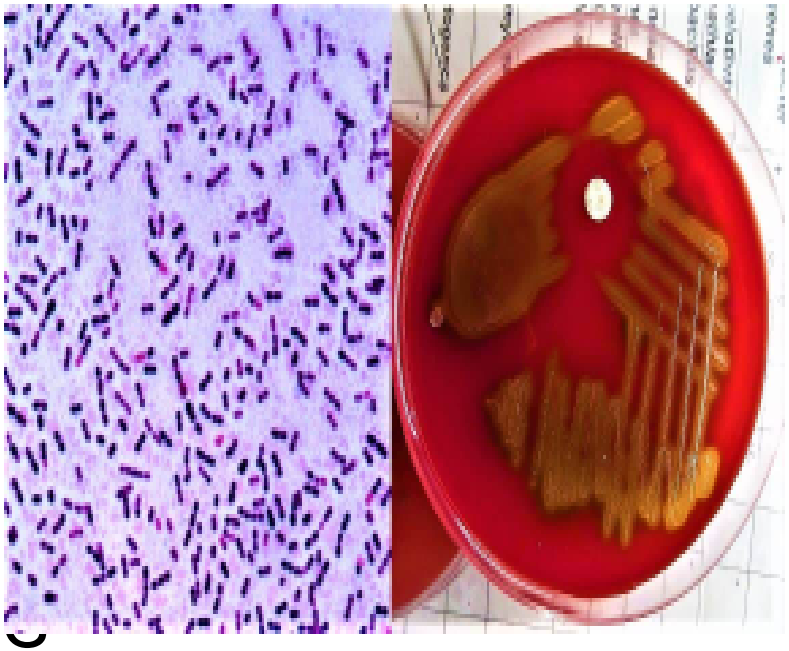
Bilateral ACUTE SUPPURATIVE OTITIS MEDIA in the stage of presuppuration with conductive hearing loss , having the source in the pharynx – Acute pharyngitis.

MANAGEMENT :

1) INVESTIGATION :

CBC

Throat swab for culture of micro-organism



On culture:

* Gram positive

* anaerobic organism

* Produces green colonies

on blood agar

TREATMENT:

1) Antibacterial therapy:

Ampicillin : 50mg/kg/day in 4 divided doses

Amoxicillin: 40 mg /kg/day in 3 divided doses

Those allergic to penicillin: cefaclor,cotrimoxazole, or erythromycin.

Antibacterial therapy must be continued for a minimum of 10 days , till TM regains normal appearance and hearing returns to normal

2) Decongestant nasal drops: To relieve ET edema and promote ventilation of middle ear

Oral nasal decongestants are preferred in children in whom administering drops will be difficult.

3) Analgesics and Antipyretics: Paracetamol helps to relieve pain and bring down temperature

4) Myringotomy


THANK
YOU